



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : الفت سعيد محمد عواد

المحافظة : القاهرة الحي / المدينة : الساحل

العنوان : 11 ش المامون
الخلاوى الساحل القاهرة

تاريخ تسجيل الإستمارة : 26-09-2026

رقم البطاقة : 26510060102747 رقم التليفون : 01116325837

العمر (تاريخ الميلاد) : 06-10-1965 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: الفت سعيد محمد عراد National ID: 26510060102747	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Contrast-enhanced MRI of the brain and orbits
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية

بطاقة تحقيق الشخصية



الفت

سعيد محمد عواد

٩١٢ غرب المطار - حي ١١ - قطاع أ

ثالث ٦ اكتوبر - الجيزه

١٩٦٥/١٠/١٢

٢ ٦٥ ١٠ ٠ ٦ ٠ ١ ٠ ٢٧ ٤٧

LU7431251



٢٠٢٦/٠٦ | ٢٦٥١٠٠٦٠١٠٢٧٤٧

حاصلة على دبلوم تجارى



انتى مسلمة متزوجة

محمود شوقي عبدالمنعم حنفى

البطاقة سارية حتى ٢٠٣٣/٠٦/٢٤



Name: الفت سعيد محمد عواد		
MR. : 9688		
Detailed Medical Report		
		السيد
تجبة طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a 60-year-old individual with a long-standing history of diabetes mellitus, currently on carbozole. The initial diagnosis was left-sided invasive lobular carcinoma, grade 2, clinical stage T4b N3, estrogen receptor 0/8, progesterone receptor 5/8, HER2 3+. Neoadjuvant chemotherapy consisted of four cycles of PHASGO combined with 10 weeks of Taxol and carboplatin; treatment was halted because of grade 3 peripheral neuropathy, after which the regimen was switched to capecitabine (Xeloda). The patient subsequently underwent left mastectomy in May 2025 and later received two additional cycles of adjuvant PHASGO. The patient did not receive a port and was lost to follow-up; the most recent imaging showed skin nodules prompting biopsy for a new CT chest, abdomen/pelvis, and bone scan. In July 2026, a tru-cut biopsy of the left breast recurrence demonstrated invasive micropapillary carcinoma with focal signet-ring features, grade 3, hormone-receptor negative, HER2 3+. Staging work-up, including bone scan and CT chest/abdomen/pelvis, revealed no distant metastases, and the case was presented at multidisciplinary tumor board (MDT). In August 2026, the MDT concluded that the patient had locoregional recurrence manifesting as multiple chest-wall nodules without distant disease. Surgical assessment determined that wide local excision was not feasible due to disseminated involvement. The treatment plan was to initiate Taxol-Herceptin therapy, with consideration of Navelbine-Herceptin if Taxol intolerance occurred, and to monitor clinical response closely. Chest-wall radiation was to be started after maximal systemic response. The regimen was documented as C1 W1 Taxol + C1 Herceptin. On the same day, the pharmacy note recorded the patient's weight (57 kg) and height (165 cm) and confirmed administration of week 1 Taxol only, with Herceptin to be given after an echocardiogram. The patient has completed week 1 of Taxol.		
تحريرا في ,		
	الوقت :	20-09-2026 التاريخ :
	mohamed ismael الطبيب المعالج :	
يعتمد ,		



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبعة
مصر

MDT FORM

4/6/2025 G

Name	الفت حبه حوار	MRN	27581224
Age	59	ID	13997049
Menopausal status	Menopausal	Phone	01220076951
NRS SCORE		Marital status	Married
BMI		Family history	5761...71.54
CO morbidity	DH	REASON OF MDT	Postoperative

Clinical Data

BX:	L+ Breast Cancer Post NACT, MRM			L
ER:	PR:	HER 2:	Ki67:	
	IDC Grade II			
Sonomammography:	Maximal tumor diameter: 3.7 cm Actual tumor cellularity: 71.7% Tumor regression: 93%			
Mets Workup:	Free deep Margin Free nipple ad Areola Focal lymphovascular invasion ad calcification			

Recommendations:

Positive Metastatic deposits 1/12

Pathological Partial Response

Stage: yPT2 yPN1

RCB: 2-83 RCB Class II

MDT OUTCOME & PLAN

- PORT

- Adjuvant

- EP, PR on testing for H-T or not

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. Key					



Patient Name	: الأستاذة / الفاتمة سعيد محمد عواد	Request No.	: 13002222
Patient No.	: 26510060102747	Request Date	: 05/05/2025
Path No.	: 1613/25	Verification Date	: 21/05/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 59 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Left MRM measured 23x14x3.5 cm with covering skin ellipse measured 18x11 cm.
- * Tumor extent: A greyish white firm tumor bed site.
- * Size: 11x9x5 cm.
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 0.4 cm away from deep margin and 0.7 cm from skin with dilated ducts oozing yellowish and brownish material.
- * Attached axilla: Fibrofatty tissue measured 5.5 x5x2 cm entangling 12 L.Ns up to 2x1 cm.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Maximal tumor diameter: 3x2.5 cm
- Actual tumor cellularity: 7%, tumor necrosis index: 93%
- * Ductal carcinoma in situ: Not detected
- * Skin involvement: Free nipple & areola.
- * Lymph node involvement: Positive metastatic deposits 1/12.
- * Resection Margins: Free deep margin
- * Microcalcifications: Present
- * Lymphovascular invasion: Focal.
- * Histologic response to therapy: Pathological partial response

Diagnosis:

Left breast cancer post NACT, MRM specimen:

Verified by :

Printing Date : 15/06/2025 14:48:19

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



خريطة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche- Min El-Qadima- Cairo, Egypt

Patient Name : الأستاذ / الفات محمد عواد

Patient No : 26510060102747

Path No : 1613/25

Sex : Female

Age : 59 Y

Referring Doc :

Request No. : 13002222

Request Date : 05/05/2025

Verification Date : 21/05/2025

Financial Category : التأمين - طبقة الدولة

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Invasive duct carcinoma , Grade II.

- Maximal tumor diameter: 3x2.5 cm

- Actual tumor cellularity: 7%, tumor necrosis index: 93%.

- Free deep margin

- Free nipple & areola

- Focal lymphovascular invasion and calcification.

- Positive metastatic deposits 1/12.

- Pathological partial response.

- Stage ypT2 ypN1

- RCB 2.83, RCB class II

Signature

Dr. Rana Fathy

Dr. Menan Hossam

Verified by :

Printing Date : 15/06/2025 14:48:19

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



جولان رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الطبية - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche- Mfwr El Qadima- Cairo, Egypt

Test ID: 1000102747	Request No: 13702951
Request Date: 26/08/2024	Request Date: 16/08/2024
Ref: SR-1100124	Verification Date: 27/08/2024
Sex: Female	Financial Category: ٢٢٠٢٤ : ٢٠٢٤ : ٢٠٢٤ : ٢٠٢٤ : ٢٠٢٤
Age: 58 Y	Department: 1001 Patient
Referring Doc:	Registered by: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer and left axillary lymph node.

Feature of Specimen: Slide Revision

Gross Examination:

- 5 paraffin blocks labeled by (4332/24) and 5 slides were received.

Microscopic Examination of slides labelled by (1&2):

- * Histological type: Intraductal proliferation with papillary apocrine changes.

Microscopic Examination of slides labelled by (A&B):

- * Histological type: Invasive duct carcinoma with microcapillary features.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
- * DCIS: Focal.

* Angiolymphatic: No.

* Perineural invasion: No.

⇒ T of slides labelled by (A&B):

* CK5/6: Negative in invasive foci.

* ER: No nuclear reaction in the tumor cells (score 0/8).

Microscopic Examination slides labelled by D (left axillary lymph node)

Histological type: Metastatic microcapillary carcinoma in lymph node.

IPT of slides labelled by (D):

- GATA3: Positive.

- PR: Moderate nuclear reaction in 30% of tumor cells (score 5/8).

- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Left breast cancer and left axillary lymph node. Slide revision

Diagnosis of slides labelled by (1&2):

Verified by

Printing Date: 24/10/2024 12:45:14

Page 1 of 2

Printing By: r.ahmed@elms



رقب ٢٢٠٢٤ : ٢٧ ٨ ٢٠٢٤ : ٢٠٢٤ : ٢٠٢٤ : ٢٠٢٤ : ٢٠٢٤
Room No: 140 - First Floor - El Mansoury Hadad - El Dokki - Cairo - Egypt

خطابات أورام

Nuclear Medicine Unit

Patient Name: Olfat Saied Mohamed

Date: 12/11/2024

Referring physician: Prof.

^{99m}Tc -MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- No definite active hot spot that could account for osseous metastatic lesions .
- Spondylodegenerative changes are seen at the dorso-lumbar spine .
- Osteoarthritic changes are seen at both shoulders , both knees .
- Both kidneys are normally seen.

Conclusion:

No definite hot spot that could account for osseous metastatic lesions .

Thanks for the referral

Nuclear Medicine Unit

DR. Sherin Mahdy, MD

DR/ Mohamed Emam



Patient Name: Olfat Saeed Mohamed Awwad	Date : 04/11/2024
Exam : C.T CHEST WITH CONTRAST	Age : 59 Year(s)
Medical ID: 2758/2024	Pat. ID: 93878

- The liver is mildly enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Opinion:

- **Reported case of breast cancer.**
- **Prominent left para-tracheal lymph nodes for close follow up.**
- **No definite pulmonary or hepatic metastatic deposits.**

Dr. Mohamed Mohsen, M.D.

جمهورية مصر العربية
وزارة الصحة و السكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة



خطاب إدارى

الرقم القومى	٢٦٥١٠٠٦٠١٠٢٧٤٧
إسم المواطن	الفت سعيد محمد عواد

٢٧٣٦٩٢٢٩	رقم الطلب	تاريخ اللجنة ٢٠٢٦-٠٨-٢٠ جهة الإرسال جوسٹاف روسي انتر نیشنل - مصر (EPE) *
خطاب إداري	الحالة	
نص الخطاب		
نواف بتقديم حديث بالأشعة و mdt واضحة مفصلة بالحالة		
		
٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٦		

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

تصويراً في ٢٠٢٠-٠٨-٢٠
٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٦

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

المسيد /

تحية طيبة وبعد،،

إيماء للطلاب المقدم من بشأن تقرير طبي للمريض: أ.العزيزي وسعيد

يقسم: رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

metastatic breast cancer

her2 +ve

for

herceptin IV / 3ul

دهيريسين (ورلي)

تحريراً في

الوقت: التاريخ: ٢٠٢٦ / ٨ / ١

الطبيب المعالج

د. دهريسين

يقسم: ،،

Patient's Name:
Date:
Referred by:
Hospital:

Olfat Sa
2026-07-26
Prof.Dr. no doctor
Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of left breast cancer with Left Radical Mastectomy with No Chemo
or Radiotherapy following the Surgery

I- CHEST:

- Noted left breast diffuse skin thickening with 2 left breast nodular masses at the surgical wound area, anterior one (3.0x1.7cm) and posterior one measuring (3.7x2.2cm) yet with no axillary lymphnodes could be seen.
- Clear both lungs showing no pulmonary nodules, masses, consolidation, collapse or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial effusion.
- Normal plain CT appearance of the heart and great vessels but with noted thoracic and abdominal aortic calcific atherosclerotic changes.
- Intact bony thoracic cage but with noted mild Vertebral spondylosis.

II- ABDOMEN AND PELVIS:

- enlarged sized liver showing diffuse hypo yet homogenous parenchymal attenuation.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
- Normal non-contrast C.T appearance of the spleen, pancreas, suparenal glands.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic sizable lymphadenopathy.
- No ascites.
- Noted lumbar Vertebral spondylosis

Opinion:

- ✓ Known left breast cancer with discontinuation of therapy after Radical Mastectomy with left breast nodular masses as described for breast Ultrasound &/or MRI correlation.
- ✓ No Detectable Chest of Abdominopelvic Nodules or masses.
- ✓ Fatty Hepatomegaly.

Best regards,

Dr. Ayman Shawky, FEBR

Patient Name	الفت ممد محمد	Case No.	276255
Sex/Age	Female 60 Y	Patient No.	20697
Registered Date	13/6/2026 10:12:27 AM	Referred By Dr.	Interventional Radiology
Validation Date	05/7/2026 03:46:21 PM	Branch	Gustave Roussy
Reporting Date	11/7/2026 12:48:30 PM	Pathology No.	1865/26

Clinical Data: Left breast mastectomy bed nodule.

Nature of Specimen: Tru-cut biopsy, collected on 13/06/2026, at 1:40 pm. Cold ischemia time is 20 seconds.

Gross Examination:
-Fragmented grey, white tissue cores, ranging in size between 0.5 cm & 1.0 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
-Histologic type: Invasive micropapillary carcinoma with focal signet ring features.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2)
- DCIS: Absent
- Microcalcification: Absent
- Angioinvasion: Absent
- Perineural invasion: Absent
- Tumor infiltrating lymphocytes (TILs): Moderate.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 65%.

Conclusion:

Left breast mastectomy bed nodule, Tru-cut biopsy:

-Invasive micropapillary carcinoma with focal signet ring features, Grade 3.
IPT:
- ER: Negative (score 0/8).



Patient Name	الفت سعاد محمد	Case No.	276255
Sex/Age	Female 60 Y	Patient No.	20697
Registered Date	13/6/2026 10:12:27 AM	Referred By Dr.	Interventional Radiology
Validation Date	05/7/2026 03:46:21 PM	Branch	Gustave Roussy
Reporting Date	11/7/2026 12:48:30 PM	Pathology No.	1865/26

- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 65%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim, MD.



خريطة رقم (4) - الدور الأول - مبنى رقم (1987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Miter El Qadema - Cairo, Egypt



NILE

SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Olfat Said Mohammed Awad

Date : 25-05-2026 08:16

Branch : Dokki

Medical No. : 382122

Ref.Physician Prof : Goustav Roussey Egypt

Tc-99m MDP SKELETAL SCINTIGRAPHY

CLINICAL HISTORY & INDICATION:

- 61-year-old female patient with history of breast cancer; referred for metastatic workup.

TECHNIQUE

- Anterior and posterior whole-body scans were acquired 2 - 3 hours post injection of 20 mCi of Tc-99m MDP.

FINDINGS

- Axial Skeleton: Normal tracer distribution throughout the skull, spine, and thoracic cage. No focal areas of pathologically increased or decreased osteoblastic activity are detected to suggest skeletal metastasis.
- Appendicular Skeleton: Symmetric physiological radiotracer uptake is noted throughout the upper and lower extremities. No focal osseous lesions are visualized.
- Soft Tissue & Kidneys: Symmetrical renal excretion is noted bilaterally. Normal physiological urinary bladder activity is visualized. Expected left-sided chest wall soft tissue asymmetry is noted secondary to the patient's surgical history (left mastectomy).

IMPRESSION

- No scintigraphic evidence of osteoblastic active osseous metastasis.
- Follow-up imaging only if clinically indicated.

Much oblige

Dr. Alaa Nagi, MD
Nuclear medicine

R E F E R E N C E C E N T R E D I A G N O S T I C N I L E



٢٠٢٢/٠٦

٢٦٥١٠٠٦٠١٠٢٧٤٧

حاصلة على بيلوم تجاري

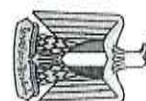
متزوجة

مسلمة

أنثى

محمود شوقي عبد المنعم حنفي

البطاقة سارية حتى ٢٠٢٢/٠٦/٢٤



١



تجارتك في كنفنا
بطاقة تحقيق الشخصية

الفت

سعيد محمد عواد

٩١٢ غرب المطرحي ١ قطاع

ثالث أكتوبر - الجيزة



١٩٦٥/٠١/٠٩

٢٦٥١٠٠٦٠١٠٢٧٤٧

LU7431251

جمهورية مصر العربية
وزارة الصحة و السكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة



خطاب إدارى

الرقم القومى	٢٦٥١٠٠٦٠١٠٢٧٤٧
إسم المواطن	الفت سعيد محمد عواد

رقم الطلب	٢٠٢٦٠٨٠٢٠	تاريخ اللجنة ٢٠٢٦٠٨٠٢٠ جهة الإرسال جوستاف روسي انتر ناثيونال - مصر (EPE) *
الحالة		
خطاب إداري		
نص الخطاب	نواف بتقييم حديث بالاشعة و mdt واضحة مفصله بالحاله	
	٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٦	

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

تصويراً في ٢٠٢٠-٠٨-٢٠
٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٦

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

المسيد /

تحية طيبة وبعد،،

إيماء للطلاب المقدم من بشأن تقرير طبي للمريض: أ.العزيزي وسعيد

يقسم: رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

metastatic breast cancer

herta +ve

for

herceptin IV / 3ul

دهيريسين (ورلي)

تحريراً في

الوقت: التاريخ: ٢٠٢٦ / ٨ / ١

الطبيب المعالج

د. د. أ.العزيزي وسعيد

يقدم ،،

Patient's Name:
Date:
Referred by:
Hospital:

Olfat Sa
2026-07-26
Prof.Dr. no doctor
Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of left breast cancer with Left Radical Mastectomy with No Chemo
or Radiotherapy following the Surgery

I- CHEST:

- Noted left breast diffuse skin thickening with 2 left breast nodular masses at the surgical wound area, anterior one (3.0x1.7cm) and posterior one measuring (3.7x2.2cm) yet with no axillary lymphnodes could be seen.
- Clear both lungs showing no pulmonary nodules, masses, consolidation, collapse or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial effusion.
- Normal plain CT appearance of the heart and great vessels but with noted thoracic and abdominal aortic calcific atherosclerotic changes.
- Intact bony thoracic cage but with noted mild Vertebral spondylosis.

II- ABDOMEN AND PELVIS:

- enlarged sized liver showing diffuse hypo yet homogenous parenchymal attenuation.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
- Normal non-contrast C.T appearance of the spleen, pancreas, suparenal glands.
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- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic sizable lymphadenopathy.
- No ascites.
- Noted lumbar Vertebral spondylosis

Opinion:

- ✓ Known left breast cancer with discontinuation of therapy after Radical Mastectomy with left breast nodular masses as described for breast Ultrasound &/or MRI correlation.
- ✓ No Detectable Chest of Abdominopelvic Nodules or masses.
- ✓ Fatty Hepatomegaly.

Best regards,

Dr. Ayman Shawky, FEBR

Patient Name	الفت سىوط محمد	Case No.	276255
Sex/Age	Female 60 Y	Patient No.	20697
Registered Date	13/6/2026 10:12:27 AM	Referred By Dr.	Interventional Radiology
Validation Date	05/7/2026 03:46:21 PM	Branch	Gustave Roussy
Reporting Date	11/7/2026 12:48:30 PM	Pathology No.	1865/26

Clinical Data: Left breast mastectomy bed nodule.

Nature of Specimen: Tru-cut biopsy, collected on 13/06/2026, at 1:40 pm. Cold ischemia time is 20 seconds.

Gross Examination:
-Fragmented grey, white tissue cores, ranging in size between 0.5 cm & 1.0 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
-Histologic type: Invasive micropapillary carcinoma with focal signet ring features.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2)
- DCIS: Absent
- Microcalcification: Absent
- Angioinvasion: Absent
- Perineural invasion: Absent
- Tumor infiltrating lymphocytes (TILs): Moderate.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 65%.

Conclusion:

Left breast mastectomy bed nodule, Tru-cut biopsy:

-Invasive micropapillary carcinoma with focal signet ring features, Grade 3.
IPT:
- ER: Negative (score 0/8).



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**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

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IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim, MD.



خريطة رقم (4) - الدور الأول - مبنى رقم (1987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Miter El Qadema - Cairo, Egypt



NILE

SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Olfat Said Mohammed Awad

Date : 25-05-2026 08:16

Branch : Dokki

Medical No. : 382122

Ref.Physician Prof : Goustav Roussey Egypt

Tc-99m MDP SKELETAL SCINTIGRAPHY

CLINICAL HISTORY & INDICATION:

- 61-year-old female patient with history of breast cancer; referred for metastatic workup.

TECHNIQUE

- Anterior and posterior whole-body scans were acquired 2 - 3 hours post injection of 20 mCi of Tc-99m MDP.

FINDINGS

- Axial Skeleton: Normal tracer distribution throughout the skull, spine, and thoracic cage. No focal areas of pathologically increased or decreased osteoblastic activity are detected to suggest skeletal metastasis.
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- Soft Tissue & Kidneys: Symmetrical renal excretion is noted bilaterally. Normal physiological urinary bladder activity is visualized. Expected left-sided chest wall soft tissue asymmetry is noted secondary to the patient's surgical history (left mastectomy).

IMPRESSION

- No scintigraphic evidence of osteoblastic active osseous metastasis.
- Follow-up imaging only if clinically indicated.

Much oblige

Dr. Alaa Nagi, MD

Nuclear medicine

R E F E R E N C E C E N T R E D I A G N O S T I C N I L E



٢٠٢٢/٠٦

٢٦٥١٠٠٦٠١٠٢٧٤٧

حاصلة على بيلوم تجاري

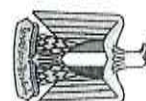
متزوجة

مسلمة

أنثى

محمود شوقي عبد المنعم حنفي

البطاقة سارية حتى ٢٠٢٢/٠٦/٢٤



جمهورية دولة فلسطين العربية
بطاقة تحقيق الشخصية

الفت

سعيد محمد عواد

٩١٢ غرب المطرحي ١ قطاع أ

ثالث ٦ أكتوبر - الجيزة



١٩٦٥/٠١/٠٨

٢٦٥١٠٠٦٠١٠٢٧٤٧

LU7431251

جمهورية مصر العربية
وزارة الصحة و السكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة



خطاب إدارى

الرقم القومى	٢٦٥١٠٠٦٠١٠٢٧٤٧
إسم المواطن	الفت سعيد محمد عواد

رقم الطلب	٢٠٢٦٠٨٠٢٠
الحالة	
خطاب إدارى	
نص الخطاب	تواف بتقييم حديث بالأشعة و mdt واضحة مفصله بالحاله ٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٦
تاريخ اللجنة	
جهة الإرسال	
جوستاف روسي انقر بالثيودال - مصر (EPE) *	

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

تصويراً في ٢٠٢٠-٠٨-٢٠
٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٦

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

المسيد /

تحية طيبة وبعد،،،

إيماء للطلاب المقدم من بشأن تقرير طبي للمريض: أ.العزيز.وسليم

يقسم: رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته:

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وبعد الكشف والفحوصات والمتابعة وجد أنه:

metastatic breast cancer

horm +ve

for

herceptin IV / 3ul

دهيريسين (ورلي)

تحريراً في

التاريخ: ٢٠٢٦ / ٨ / ١

الوقت:

الطبيب المعالج

د. أ.العزيز.وسليم

يقسم: ،،

Patient's Name:
Date:
Referred by:
Hospital:

Olfat Sa
2026-07-26
Prof.Dr. no doctor
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Conclusion:

Left breast mastectomy bed nodule, Tru-cut biopsy:

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خريطة رقم (4) - الدور الأول - مبنى رقم (1987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Miter El Qadema - Cairo, Egypt



NILE

SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Olfat Said Mohammed Awad

Date : 25-05-2026 08:16

Branch : Dokki

Medical No. : 382122

Ref.Physician Prof : Goustav Roussey Egypt

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Much oblige

Dr. Alaa Nagi, MD

Nuclear medicine

R E F E R E N C E C E N T R E D I A G N O S T I C N I L E



٢٠٢٢/٠٦

٢٦٥١٠٠٦٠١٠٢٧٤٧

حاصلة على بيلوم تجاري

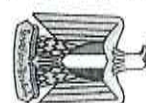
متزوجة

مسلمة

أنثى

محمود شوقي عبد المنعم حنفي

البطاقة سارية حتى ٢٠٢٢/٠٦/٢٤



١



تجارتك في كفة الميزان
بطاقة تحقيق الشخصية

الفت

سعيد محمد عواد

٩١٢ غرب المطرحي ١ قطاع

ثالث أكتوبر - الجيزة



١٩٦٥/٠١/٠٩

٢٦٥١٠٠٦٠١٠٢٧٤٧

LU7431251

جمهورية مصر العربية
وزارة الصحة و السكان
المجالس الطبية المتخصصة
الشبكة القومية لعلاج المواطنين على نفقة الدولة



خطاب إدارى

الرقم القومى	٢٦٥١٠٠٦٠١٠٢٧٤٧
إسم المواطن	الفت سعيد محمد عواد

٢٧٣٦٩٢٢٩	رقم الطلب	تاريخ اللجنة ٢٠٢٦-٠٨-٢٠ جهة الإرسال جوستاف روسي انتر ناشيونال - مصر (EPE) *
خطاب إداري	الحالة	
نص الخطاب		
نواف بتقديم حديث بالأشعة و mdt واضحة مفصلة بالحالة		
٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٦		

د/ محمد عقاد
الإدارة العامة للمجالس الطبية المتخصصة
مدير عام

تصويراً في ٢٠٢٠-٠٨-٢٠
٢٢٧١٤ / ٣٩٢٥ / ٢٨٦٦٦

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

المسيد /

تحية طيبة وبعد،،

إيماء للطلاب المقدم من بشأن تقرير طبي للمريض: أ.العزيزي

يقسم: رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته:

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد أنه:

metastatic breast cancer

hert2 +ve

for

herceptin IV / 3ul

دهيريسين (ورلي)

تحريراً في

الوقت: التاريخ: ٢٠٢٦ / ٨ / ١

الطبيب المعالج

د. دهريسين

يقدم ،،

Patient's Name: Olfat Sa
Date: 2026-07-26
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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- Ki67%: 65%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim, MD.



خريطة رقم (4) - الدور الأول - مبنى رقم (1987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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NILE

SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Olfat Said Mohammed Awad

Date : 25-05-2026 08:16

Branch : Dokki

Medical No. : 382122

Ref.Physician Prof : Goustav Roussey Egypt

Tc-99m MDP SKELETAL SCINTIGRAPHY

CLINICAL HISTORY & INDICATION:

- 61-year-old female patient with history of breast cancer; referred for metastatic workup.

TECHNIQUE

- Anterior and posterior whole-body scans were acquired 2 - 3 hours post injection of 20 mCi of Tc-99m MDP.

FINDINGS

- Axial Skeleton: Normal tracer distribution throughout the skull, spine, and thoracic cage. No focal areas of pathologically increased or decreased osteoblastic activity are detected to suggest skeletal metastasis.
- Appendicular Skeleton: Symmetric physiological radiotracer uptake is noted throughout the upper and lower extremities. No focal osseous lesions are visualized.
- Soft Tissue & Kidneys: Symmetrical renal excretion is noted bilaterally. Normal physiological urinary bladder activity is visualized. Expected left-sided chest wall soft tissue asymmetry is noted secondary to the patient's surgical history (left mastectomy).

IMPRESSION

- No scintigraphic evidence of osteoblastic active osseous metastasis.
- Follow-up imaging only if clinically indicated.

Much oblige

Dr. Alaa Nagi, MD
Nuclear medicine

R E F E R E N C E C E N T R A L D I A G N O S T I C



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حاصلة على بيلوم تجاري

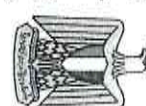
متزوجة

مسلمة

أنثى

محمود شوقي عبد المنعم حنفي

البطاقة سارية حتى ٢٠٢٢/٠٦/٢٤



جمهورية دولة فلسطين العربية
بطاقة تحقيق الشخصية

الفت

سعيد محمد عواد

٩١٢ غرب المطرحي ١ قطاع أ

ثالث ٦ أكتوبر - الجيزة



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